

Question:

A 5-year-old boy is brought to the physician by his mother because he does not “listen to her” anymore. The mother also reports that her son cannot concentrate on any tasks lasting longer than just a few minutes. Teachers at his preschool report that the patient is more active compared to other preschoolers, frequently interrupts or bothers other children, and is very forgetful. Last year the patient was expelled from another preschool for hitting his teacher and his classmates when he did not get what he wanted and for being disruptive during classes. He was born at term via vaginal delivery and has been healthy except for 3 episodes of acute otitis media at the age of 2 years. He has met all developmental milestones. His mother has major depressive disorder and his father has Graves' disease. He appears healthy and well nourished. Examination shows that the patient does not seem to listen when spoken to directly. The remainder of the examination shows no abnormalities. Which of the following is the most appropriate next step in treatment?

Answer Choices:

- A. Behavior therapy
- B. Methimazole
- C. Fluoxetine
- D. Hearing aids

Reasoning + Answer:

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Finding reasoning paths :

- 1 . Short attention span and difficulty sustaining focus -> Possible attention deficit hyper activity disorder (ADHD) -> Treatment with stim ulant medications or behavioral therapy .
- 2 . Impulsivity and disruptive behavior -> Consider ation of ADHD or opposition al defiant disorder (OD D) -> Behavioral therapy and/or medication .
- 3 . Forgetfulness and inattentiveness -> Possible ADHD or other learning disorders -> Behavioral therapy and/or medication .

Reasoning Process :

- 1 . The child exhibits a short attention span , difficulty sustaining focus , impulsivity , and disruptive behavior . These symptoms are characteristic of ADHD , which is commonly diagnosed in young children presenting with these behaviors .
- 2 . The absence of developmental delays and the presence of a normal developmental history suggest that the symptoms are not due to a neurological or genetic syndrome but rather a behavioral or attention-related issue .
- 3 . The mother's major depressive disorder could potentially influence the child's behavior , but the specific symptoms of inattention and hyperactivity are more consistent with ADHD than with symptoms of depression affecting behavior .
- 4 . Behavioral therapy is often the first-line treatment for ADHD , especially in younger children , as it addresses both the child's behavior and teaches strategies for improved attention and impulse control .
- 5 . If symptoms persist or worsen , medication such as methyl phen idate may be considered , but initial steps typically involve behavioral interventions .

Conclusion :

Behavior therapy is the most appropriate next step in treatment for this 5-year-old boy , addressing his short attention span , impulsivity , and disruptive behavior , consistent with a diagnosis of attention deficit hyper activity disorder .

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< answer >

Behavior therapy

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